

BRIGHTPATH HEALTH CLINIC

Business Operations Audit

An independent diagnostic of the clinic group's operational readiness, revenue leakage and scale-up priorities.

PREPARED FOR

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DATE

21 August 2026

CONFIDENTIAL · EDUCATIONAL CASE ANALYSIS

Brightpath has demand. Its operating system is the constraint.

The case evidence points to a healthy customer proposition being held back by fragmented workflows, disconnected records and limited management visibility.

5

clinics across the West Midlands

25

employees across clinical + support roles

~£2m

annual revenue in the case brief

Audit conclusion

Brightpath's strongest assets are its five-star reputation, founder credibility, broad service mix and strong word-of-mouth. The immediate risk is not lack of demand; it is the ability of the internal operating model to absorb more demand without creating more errors, administrative load or staff pressure.

WHAT IS WORKING

Patient experience is strong; the business has a diversified offer across physiotherapy, nutrition, mental-health therapy and wellness; the founder brings clinical credibility; and the brand already benefits from organic referrals.

WHAT IS HOLDING GROWTH BACK

Scheduling is fragmented, patient histories are separated by location, marketing activity is difficult to measure, clinics operate as silos, and leadership lacks a live performance view.

Priority finding

The first intervention should be a group-wide scheduling layer. It addresses the most visible daily friction—double-bookings, unreliable availability and inconsistent confirmations—and creates the operational foundation for capacity management across all five sites.

Estimated annual opportunity: the reference audit model places the combined cost of the five inefficiencies at approximately **£112,000 per year**. These are planning estimates rather than audited accounts and should be validated with Brightpath's own records before investment decisions.

A fast-growing five-site healthcare business

The audit uses the supplied Brightpath case brief as its factual baseline and treats the operational gaps as a connected system rather than isolated problems.

BUSINESS	CASE DETAIL
Industry	Private healthcare: physiotherapy, nutrition coaching, mental-health therapy and wellness programmes
Footprint	Five clinics: Birmingham, Wolverhampton, Coventry, Solihull and Walsall
People	25 employees: clinicians, receptionists, one office manager and one marketing freelancer
Founder	Dr. Amara Okonkwo, a GP who moved into wellness entrepreneurship
Stage	Growing quickly; considering two additional cities but not yet operationally ready

Founder signal

The case brief describes a founder spending much of the week firefighting rather than running the business. Internal processes are reportedly held together by WhatsApp groups, spreadsheets and goodwill. That is a classic scale-up warning: the organisation is relying on individual effort where repeatable systems are needed.

Diagnostic chain

1	Fragmented workflows	Separate calendars, physical files and location-level practices create inconsistency.
2	Operational friction	Staff spend time reconciling information instead of serving patients or managing capacity.
3	Commercial leakage	No-shows, rework, missed cross-sell opportunities and avoidable referrals reduce realised revenue.
4	Scale risk	Adding locations before fixing the operating model would multiply coordination problems.

Where value is leaking from the current model

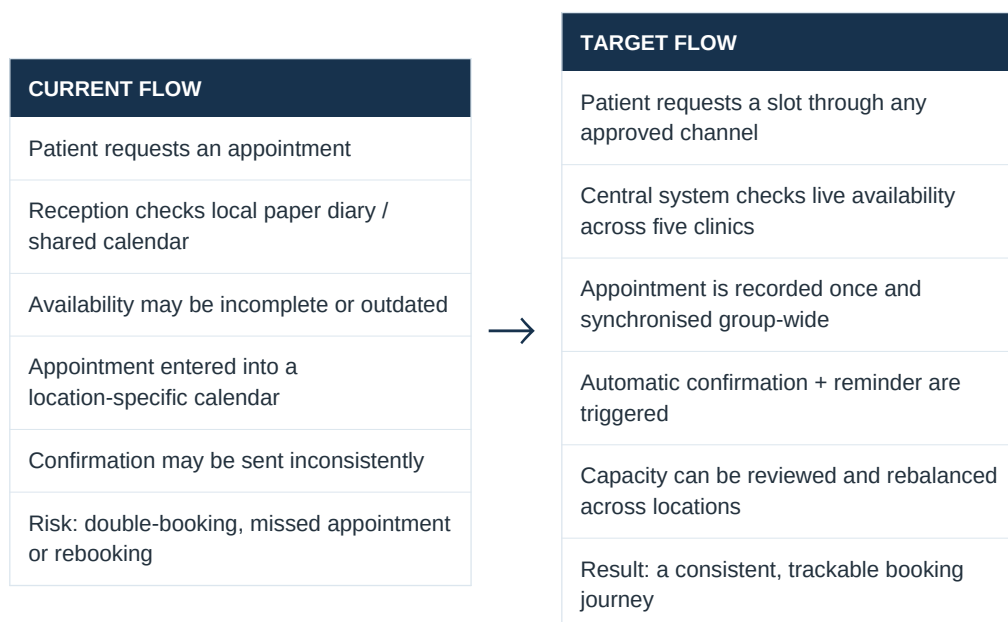
The estimates below follow the reference audit's planning assumptions while the wording and presentation are original. They should be validated against Brightpath records.

01	Scheduling fragmentation	MEDIUM	≈ £40k / year
Current-state issue		Recommended response	
Three sites rely on paper diaries while two use a shared Google Calendar that is not reliably maintained. This creates double-bookings, incorrect availability and avoidable rebooking work.		Consolidate all locations into one cloud scheduling environment with live availability, automated confirmations/reminders and cross-site capacity visibility.	
Cost logic			
Planning estimate: £36k from no-shows/rebookings (£3k × 12), plus roughly £4k of reception coordination time.			
02	Disconnected patient records	HIGH	≈ £25k / year
Current-state issue		Recommended response	
Each clinic maintains its own physical files. Patients moving between services can have their history recreated, increasing clinician rework and weakening continuity.		Introduce one GDPR-compliant practice-management system as the shared patient record, linked to the central scheduling layer.	
Cost logic			
Planning estimate: ≈£12k clinician time from duplicated intakes plus ≈£13k of missed cross-sell value; wider compliance risk is not priced.			
03	Inconsistent marketing operations	LOW	≈ £13k / year
Current-state issue		Recommended response	
Five separate Instagram accounts, inconsistent branding and no reliable content/performance calendar make the freelancer a single point of failure.		Use one content workflow, a shared brand library, scheduled publishing and UTM-based tracking; use AI-assisted drafting for routine content.	
Cost logic			
Planning estimate: ≈£5.8k lost conversions + ≈£2.9k freelancer inefficiency + ≈£5k untracked/misaligned spend.			

04	Cross-clinic coordination gap	LOW	≈ £11–14k / year
Current-state issue Sites do not have a shared view of demand and spare capacity. Patients can be referred externally while another Brightpath clinic has an available slot. Cost logic Planning estimate: around six avoidable external referrals/month × £150 average value × 12 ≈ £10.8k, with upside from multi-session care.		Recommended response Create a single internal collaboration channel, publish a daily group capacity view and establish simple routing rules for referrals.	
05	No management dashboard	MEDIUM	≈ £20k / year
Current-state issue Leadership cannot see bookings, retention or service mix in real time; revenue information arrives weeks later through the accountant. Cost logic Planning estimate: a conservative 1% revenue drift on ≈£2m = £20k/year; strategic expansion risk is not priced.		Recommended response Build a lightweight dashboard from the PMS/scheduling data covering bookings, no-show rate, revenue by clinic, retention, NPS and staff utilisation.	

From fragmented booking to one operating flow

The scheduling problem is the clearest example of how a small process change can remove several downstream failure points.



Why this change matters

Scheduling is not just a diary problem. It is the point where demand, clinician availability, location capacity, patient communication and revenue meet. A centralised workflow therefore becomes the foundation for the later dashboard and cross-clinic routing work.

Control points to introduce

CONTROL	
	One live view of clinician and room capacity across all five sites.
Confirmation	Every appointment receives the same confirmation and reminder sequence.
	Cancellations, no-shows and rebookings are recorded consistently.
Capacity	Staff can see where demand is high and where spare capacity exists.
	Booking and attendance data can feed the management dashboard.

Sequence the fixes so the business can scale with control

The recommended order balances business impact, implementation effort and dependency between systems.

PRIORITY	INITIATIVE	IMPACT	EFFORT	TIMING
1	Central scheduling + automated reminders	Very high	Medium	Month 1
2	Shared patient-management system	High	High	Months 1–2
3	Management KPI dashboard	High	Medium	Month 2
4	Marketing operating system	Medium	Low	Months 3–4
5	Cross-clinic communication + routing	Medium	Low	Months 3–6

90-day transformation sequence

PHASE 1 · 0–30 DAYS	PHASE 2 · 31–60 DAYS	PHASE 3 · 61–90 DAYS
Stabilise: select and configure the central scheduler; standardise appointment statuses; start PMS implementation; define ownership for each workflow.	Instrument: connect operational data; launch the KPI dashboard; establish a weekly management review; test the new patient-record workflow.	Optimise: centralise marketing planning; introduce cross-clinic routing; formalise SOPs; use performance data to identify remaining gaps.

Bottom line

Brightpath should treat operational readiness as a prerequisite to expansion. The case indicates approximately £112,000 of annual value at risk across the five gaps, while the reference audit estimates around £12,000 for first-year software and setup and a payback period of under three months. Those financial figures are planning estimates, not audited results. The immediate management objective should be to validate them, fix the scheduling bottleneck, establish a single data foundation, and only then commit confidently to a sixth or seventh location.

Audit basis: Brightpath Health Clinic case brief supplied with this assignment, together with the uploaded reference Business Operations Audit template. The business is fictional and intended for educational use.